

Chemotherapy for cervical cancer

Chemotherapy may be your main treatment. Or it may be given to reduce the risk of cancer coming back after surgery.

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What is chemotherapy?

Chemotherapy uses anti-cancer (cytotoxic) drugs to destroy cancer cells. These drugs disrupt the way cancer cells grow and divide, but they also affect normal cells.

You may have chemotherapy to treat [cervical cancer](#):

- with [radiotherapy](#) as your main treatment – this is called [chemoradiation](#)
- after surgery and with radiotherapy (chemoradiation), if there is a high risk of the cancer coming back
- if the cancer comes back after treatment
- if the cancer spreads to other parts of your body.

How chemotherapy is given

You may have chemotherapy in the chemotherapy day unit or on the ward. The drugs are usually given into a vein (intravenously).

During one or more sessions of treatment. The length of each session depends on the combination of chemotherapy drugs you have. After each session, you have a rest period. The chemotherapy session and the rest period are called a cycle of treatment.

The length of a cycle depends on the chemotherapy drugs you have, but most cycles are 1 to 3 weeks long. Your doctor or nurse will tell you how many cycles of treatment you will have.

Chemotherapy drugs for cervical cancer

A number of different drugs may be used to treat cervical cancer. Your doctor will explain to you which drugs are best for you. You may be given one chemotherapy drug, or two or more in combination.

Drugs that may be used include:

- [cisplatin](#)
- [5-fluorouracil \(5-FU\)](#)
- [doxorubicin](#) or [pegylated liposomal doxorubicin](#)
- [paclitaxel](#)
- [topotecan](#)
- [carboplatin](#)
- [gemcitabine](#).

Side effects

Chemotherapy may cause [side effects](#). For example, many drugs can make you more likely to get an [infection](#). And some drugs may affect the nerves and cause numb, tingling or painful hands or feet ([peripheral neuropathy](#)). If you know the name of the chemotherapy you are having you can search for it in our [treatment & drugs A-Z](#).

Many side effects can be controlled with medicines, and usually go away when treatment finishes. Your doctor or nurse will tell you more about what to expect. Always tell them about any side effects you have, as there are usually ways they can help.

About our information

References



Below is a sample of the sources used in our cervical cancer, cervical screening and CIN information. If you would like more information about the sources we use, please contact us at canцерinformationteam@macmillan.org.uk

GOV.UK. Cervical screening: programme overview. Updated 18 November 2019. Available from www.gov.uk/guidance/cervical-screening-programme-overview (accessed March 2020).

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Marth C, Landoni F, Mahner S, et al. Cervical cancer: ESMO clinical practice guidelines. *Annals of Oncology*, 2017; 28, suppl 4, iv72–iv83. Available from <https://www.esmo.org/guidelines/esmo-clinical-practice-guideline-cervical-cancer> (accessed October 2020).

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Professor Nick Reed](#), Consultant Clinical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 April 2021 | Next review: 01 April 2024

⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)